



CA Anti-Discrimination

..... Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens takes pride in providing equal housing opportunity for seniors. This policy underscores for all employees our commitment to anti-discrimination practices in our communities.

Guidelines

Merrill Gardens admits and retains residents without regard to race, color, national origin, ancestry, creed, sex, actual or perceived sexual orientation, actual or perceived gender identity, veteran's status, religion, marital status, registered domestic partner status or any other basis prohibited by law. Additionally, Merrill Gardens does not discriminate based upon the presence of any sensory, mental, or physical disability as long as required services are consistent with the community's licensing, available services, policies and procedures.

Reporting Discrimination

All team members must report incidents of suspected discrimination to the next level of management. Team members may exercise their rights to raise concerns and file reports without fear of reprisal. The chain of notification is as follows:

- Immediate Supervisor
- General Manager
- Vice President of Operations
- Seattle Office: Vice President of Community Operations

CA Death with Dignity

..... Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

It is the policy of this community to allow residents the freedom to exercise their right under the California End of Life Option Act (Health and Safety Code Section 1. Part 1.85 Section 443) (the Act). The process will be conducted in accordance with applicable state statutes, regulations and community policies. However, at no time, in any part of the process, will the community or its team members (employees), volunteers or contractors actively participate in the process for residents in our community, including but not limited to:

- Serving as a witness to any written request by a resident for medication that the resident may self-administer to end his or her life under the Act;
- Attending or otherwise being present during and after the injection or ingestion of a lethal dose of medication, except in the delivery of comfort care services and outlined in the resident's service plan;
- Taking possession of the medication for storage, delivery or disposal.

Guidelines

Notification

As part of the admission process, all new residents will be informed of the community policy of non-participation and asked to sign an acknowledgement form.

Team members and Volunteers will be informed of the non-participation policy during orientation.

Contractors will be informed of the non-participation policy by a notice placed in front of the sign-in log kept in the reception area.

Team members who violate this policy are subject to corrective action up to and including termination of employment. Volunteers who violate this policy will be prohibited from volunteering and may be restricted from the property. Contractors who violate this policy may be restricted from the property and their relationship terminated.

Procedure

Upon awareness of a resident's intent to exercise their right to request a medication for the purpose of ending his or her life the community will proceed as follows:

1. If a resident is making a written request for medication under the Act, the Act requires that they must have at least two witnesses, who in the presence of the resident, must attest that the

resident has the capacity to make medical decisions, is acting voluntarily and is not being coerced to make or sign the request. Accordingly, Merrill Gardens chooses not to make this attestation, and will communicate to the resident why the attestation is not being made. In addition, at least one of the witnesses must not be a relative of the resident by blood, marriage, registered domestic partnership or be entitled to inherit any portion of the resident's estate upon death.

2. Under no circumstances will the owner, operator, or any employee or agent of the community serve as a witness to any written request by a resident for medication that the resident may self-administer to end his or her life under the Act.
3. The community will conduct an immediate service plan review with the resident (and the resident's designated representative or family member upon consent of the resident) to discuss the following:
 - Encouragement to involve the assistance of a hospice agency;
 - Limitations of the community's involvement in all aspects of the aid-in-dying process;
 - Review of the resident's Advance Directives/CPR status/POLST form for any changes and help ensure they are consistent with the resident's request for aid-in-dying;
 - Documentation addressing the aid-in-dying request that should be provided to the community;
 - Where the dying process will occur (not in a public place);
 - Who the resident has designated to be present when the ingestion of the medication occurs (community team members, volunteers and contractors will not be allowed under any circumstances to be designated or otherwise present, except for the delivery of comfort care services as outlined in the service plan).
4. Help ensure the resident's service plan is updated to reflect the following:
 - Resident's desire to exercise the right to aid-in-dying;
 - The name and contact information of who the resident has designated to be present during the dying process (community team members, volunteers and contractors will not be allowed under any circumstances to be designated or otherwise present, except for the delivery of comfort care services as outlined in the service plan);
 - Resident's preferred comfort measures during the dying process.
5. The community will attempt to obtain and maintain, in the resident's file:
 - Documentation from the attending physician indicating that all requirements under the Aid In Dying Act have been met, including a notation of the aid-in-dying drug prescribed.
6. Help ensure the resident is informed that the lethal dose of medication, once obtained, must be kept off the premises or secured in a locked cabinet in the resident's room at all times until it is to be used by the resident. The lethal dose of medication shall only be accessible to the resident, his or her designated representative or family member or attending physician.
7. Community team members, volunteers and contractors will not take possession of the medication for storage, delivery or disposal.

8. If at any time following the ingestion of the lethal dose of medication, medical treatment is requested by the resident, the community's involvement will be limited to calling 911 on behalf of the resident, or his or her representative or family member. We believe that the End of Life Option Act supersedes the RCFE requirement to call 911, and therefore team members are not required to call 911 if they see a resident dying who has informed the community of their aid-in-dying plan. 911 will need to be called if a team member sees the resident dying and does not know that the resident has ingested aid-in-dying medication.
9. Under no circumstances will community team members, volunteers and contractors be allowed or required to attend or otherwise be present during and after the ingestion of a lethal dose of medication, except for the delivery of comfort care services as outlined in the service plan.

Important Notice

Regarding the California End of Life Option Act.

The End of Life Options Act (EOLOA) has been suspended for the time being due to a ruling from a Riverside County judge that deemed the Act unconstitutional. The law was passed during a special session and the argument is that the special session was meant for another purpose, making the law unconstitutional. The state attorney general asked an appeals court to block the decision and allow the provisions of the Act to continue, but the request was denied.

Currently, the state is operating as it did before the Act took effect in June 2016. Individuals will not be able to get the prescription necessary to end their lives in this manner, and for those already in possession of the drugs who planned to take the medications, their deaths would be considered suicides if carried out.

What this means is that at this time, Residents will not be permitted to end their life under the End of Life Options Act and our policy is on hold.

Please see the General Manager if you have any questions.

End of Life Option Act Acknowledgement:

Merrill Gardens will allow residents in our community the freedom to exercise their right under the California End of Life Option Act.

The resident must exercise this right through an attending physician in accordance with applicable laws, regulations and policies.

Team Members, Volunteers, Contractors and Outside Providers may NOT participate in any part of the aid-in-dying process including, but not limited to:

- Serving as a witness to any written request made by a resident for aid-in-dying medication,
- Attending or otherwise being present during and after the self-administration of a lethal dose of medication, except in the delivery of comfort care services as outlined on the resident's service plan.
- Taking possession of the medication for storage, delivery or disposal.

Team members who violate this policy are subject to corrective action up to and including termination of employment. Volunteers who violate this policy will be prohibited from volunteering and may be restricted from the property. Contractors/Outside Providers who violate this policy may be restricted from the property and their relationship terminated.

The resident is not required to notify the community of their desire to exercise their right, but if the community is made aware, we will review any Advance Directives/POLST forms to help ensure they are consistent with the resident's plan and update the Service Plan with the resident's preferences for comfort care.

Please see the General Manager if you have any questions.

By signing this document, the resident is acknowledging that they have been made aware of the community policy of non-participation in the aid-in-dying process.

Resident Signature

Printed Name

Date

Notice to all Volunteers, Contractors and Outside Providers:

Merrill Gardens will allow residents in our community the freedom to exercise their right under the California End of Life Option Act.

Volunteers, Contractors and Outside Providers may NOT participate in any part of the aid-in-dying process including, but not limited to:

- Serving as a witness to any written request made by a resident for aid-in-dying medication,
- Attending or otherwise being present during and after the self-administration of a lethal dose of medication, except in the delivery of comfort care services as outlined on the resident's service plan.
- Taking possession of the medication for storage, delivery or disposal.

Volunteers who violate this policy will be prohibited from volunteering and may be restricted from the property.

Contractors/Outside Providers who violate this policy may be restricted from the property and their relationship terminated.

Please see the General Manager if you have any questions.



CA Family Councils (Existing Council)

..... Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens supports the ongoing activity of a Family Council. The purpose of these meetings is to discuss issues, improve communication, and to work together to improve the quality of life for all residents. The Family Council meeting is conducted in accordance with applicable state laws. Under California law, every licensed residential care facility for the elderly, at the request of a member of a resident's family or a resident representative, must allow formation of a family council.

Guidelines

Community Has an Existing Family Council

This Community does have an existing Family council:

- The Family Council will consist of family members, friends, representatives, or agents as defined by Medi-Cal of two or more residents of the Facility.
- Merrill Gardens will provide written information to family members and resident representatives who are identified in the Admission Agreement during the admission process of the existence of the Family Council, the time and place of meetings, and the name of the Family Council representative. Merrill Gardens will provide the same information to family members and resident representatives of existing residents (as of the effective date of this policy) who are identified in the resident's records.
- Merrill Gardens will include notice of the Family Council and its meetings to family members and resident representatives in routine mailings.
- When requested by a family member or resident representative, Merrill Gardens will allow the Family Council to meet in a common meeting room at the Community during mutually agreed upon hours.
- Merrill Gardens will provide adequate space on a prominent bulletin board or other posting area at the Community for the display of meeting notices, minutes, information, and newsletters.
- Merrill Gardens will allow the Family Council to confer in private outside the presence of the Community team members.

- Merrill Gardens will not limit the right of residents and participants in the Family Council to meet independently with outside persons, including members of nonprofit or government organizations or with the Community's personnel during nonworking hours.
- Merrill Gardens' personnel or visitors may attend a Family Council meeting only at the Family Council's invitation.
- If the Family Council submits written concerns or recommendations, The General Manager will respond in writing within 14 calendar days regarding any action or inaction taken as a result.
- Upon request, and with the permission of the Family Council, Merrill Gardens will share the name and contact information of the designated representative of the Family Council with the long-term care ombudsman program.
- Merrill Gardens will appoint a designated staff liaison at the Community who will be responsible for providing assistance to the Family Council and for responding to written requests that result from Family Council meetings.
- Merrill Gardens will not willfully interfere with the formation, maintenance, or promotion of the Family Council, or its participation in the regulatory inspection process. For the purposes of this subdivision, willful interference includes, but is not limited to, discrimination or retaliation in any way against an individual as a result of his or her participation in the Family Council, refusal to publicize Family Council meetings or provide appropriate space for meetings or postings as set forth in this Policy and Procedure, or failure to respond to written requests by the Family Council in a timely manner.

CA Family Councils (No Current Council)

..... Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens supports the ongoing activity of a Family Council. The purpose of these meetings is to discuss issues, improve communication, and to work together to improve the quality of life for all residents. The Family Council meeting is conducted in accordance with applicable state laws. Under California law, every licensed residential care facility for the elderly, at the request of a member of a resident's family or a resident representative, must allow formation of a family council. the community or a secured area without supervision or the knowledge of staff.

Guidelines

Community Does Not Presently Have a Family Council

This Community does not presently have a family Council. Merrill Gardens will provide written information to family members and resident representatives who are identified in the Admission Agreement during the admission process of their right to form a family council.

Merrill Gardens will not willfully interfere with the formation, maintenance, or promotion of the Family Council, or its participation in the regulatory inspection process.

PHYSICIAN'S REPORT FOR RESIDENTIAL CARE FACILITIES FOR THE ELDERLY (RCFE)**FACILITY INFORMATION** *(To be completed by the licensee/designee)*

1. NAME OF FACILITY		2. TELEPHONE ()
3. ADDRESS	CITY	ZIP CODE
4. LICENSEE'S NAME	5. TELEPHONE ()	6. FACILITY LICENSE NUMBER

II. RESIDENT/PATIENT INFORMATION *(To be completed by the resident/resident's responsible person)*

1. NAME	2. BIRTH DATE	3. AGE
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III. AUTHORIZATION FOR RELEASE OF MEDICAL INFORMATION*(To be completed by resident/resident's legal representative)*

I hereby authorize release of medical information in this report to the facility named above.

1. SIGNATURE OF RESIDENT AND/OR RESIDENT'S LEGAL REPRESENTATIVE

2. ADDRESS	3. DATE
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IV. PATIENT'S DIAGNOSIS *(To be completed by the physician)*

NOTE TO PHYSICIAN: The person named above is either a resident or prospective resident of a residential care facility for the elderly licensed by the Department of Social Services. The license requires the facility to provide primarily non-medical care and supervision to meet the needs of that person. THESE FACILITIES DO NOT PROVIDE SKILLED NURSING CARE. The information that you provide about this person is required by law to assist in determining whether the person is appropriate for care in this non-medical facility. It is important that all questions be answered.

(Please attach separate pages if needed.)

1. DATE OF EXAM	2. SEX	3. HEIGHT	4. WEIGHT	5. BLOOD PRESSURE
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6. TUBERCULOSIS (TB) TEST

a. Date TB Test Given	b. Date TB Test Read	c. Type of TB Test	d. Please Check if TB Test is: ☐ Negative ☐ Positive
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e. Results: mm _____ f. Action Taken (if positive): _____

g. Chest X-ray Results: _____

n. Please Check One of the Following:

☐ Active TB Disease ☐ Latent TB Infection ☐ No Evidence of TB Infection or Disease

7. PRIMARY DIAGNOSIS:

- a. Treatment/medication (type and dosage)/equipment:

- b. Can patient manage own treatment/medication/equipment? ☐ Yes ☐ No
- c. If not, what type of medical supervision is needed?

8. SECONDARY DIAGNOSIS(ES):

- a. Treatment/medication (type and dosage)/equipment:

- b. Can patient manage own treatment/medication/equipment? ☐ Yes ☐ No
- c. If not, what type of medical supervision is needed?

9. CHECK IF APPLICABLE TO 7 OR 8 ABOVE:

- ☐ **Mild Cognitive Impairment:** Refers to people whose cognitive abilities are in a “conditional state” between normal aging and dementia.
- ☐ **Dementia:** The loss of intellectual function (such as thinking, remembering, reasoning, exercising judgement and making decisions) and other cognitive functions, sufficient to interfere with an individual's ability to perform activities of daily living or to carry out social or occupational activities.

10. CONTAGIOUS/INFECTIOUS DISEASE:

- a. Treatment/medication (type and dosage)/equipment:

- b. Can patient manage own treatment/medication/equipment? ☐ Yes ☐ No
- c. If not, what type of medical supervision is needed?

11. ALLERGIES:

a. Treatment/medication (type and dosage)/equipment:

b. Can patient manage own treatment/medication/equipment? ☐ Yes ☐ No

c. If not, what type of medical supervision is needed?

12. OTHER CONDITIONS:

a. Treatment/medication (type and dosage)/equipment:

b. Can patient manage own treatment/medication/equipment? ☐ Yes ☐ No

c. If not, what type of medical supervision is needed?

13. PHYSICAL HEALTH STATUS	YES	NO	ASSISTIVE DEVICE (If applicable)	EXPLAIN
a. Auditory Impairment				
b. Visual Impairment				
c. Wears Dentures				
d. Wears Prosthesis				
e. Special Diet				
f. Substance Abuse Problem				
g. Use of Alcohol				
h. Use of Cigarettes				
i. Bowel Impairment				
j. Bladder Impairment				
k. Motor Impairment/Paralysis				
l. Requires Continuous Bed Care				
m. History of Skin Condition or Breakdown				

14. MENTAL CONDITION	YES	NO	EXPLAIN
a. Confused/Disoriented			
b. Inappropriate Behavior			
c. Aggressive Behavior			
d. Wandering Behavior			
e. Sundowning Behavior			
f. Able to Follow Instructions			
g. Depressed			
h. Suicidal/Self-Abuse			
i. Able to Communicate Needs			
j. At Risk if Allowed Direct Access to Personal Grooming and Hygiene Items			
k. Able to Leave Facility Unassisted			
15. CAPACITY FOR SELF-CARE	YES	NO	EXPLAIN
a. Able to Bathe Self			
b. Able to Dress/Groom Self			
c. Able to Feed Self			
d. Able to Care for Own Toileting Needs			
e. Able to Manage Own Cash Resources			
16. MEDICATION MANAGEMENT	YES	NO	EXPLAIN
a. Able to Administer Own Prescription Medications			
b. Able to Administer Own Injections			
c. Able to Perform Own Glucose Testing			
d. Able to Administer Own PRN Medications			
e. Able to Administer Own Oxygen			
f. Able to Store Own Medications			

17. AMBULATORY STATUS:

a. 1. This person is able to independently transfer to and from bed: ☐ Yes ☐ No

2. For purposes of a fire clearance, this person is considered:

☐ Ambulatory ☐ Nonambulatory ☐ Bedridden

Nonambulatory: A person who is unable to leave a building unassisted under emergency conditions. It includes any person who is unable, or likely to be unable, to physically and mentally respond to a sensory signal approved by the State Fire Marshal, or to an oral instruction relating to fire danger, and/or a person who depend upon mechanical aids such as crutches, walkers, and wheelchairs.

Note: A person who is unable to independently transfer to and from bed, but who does not need assistance to turn or reposition in bed, shall be considered non-ambulatory for the purposes of a fire clearance.

Bedridden: For the purpose of a fire clearance, this means a person who requires assistance with turning or repositioning in bed.

b. If resident is nonambulatory, this status is based upon:

☐ Physical Condition ☐ Mental Condition ☐ Both Physical and Mental Condition

c. If a resident is bedridden, check one or more of the following and describe the nature of the illness, surgery or other cause:

☐ Illness: _____

☐ Recovery from Surgery: _____

☐ Other: _____

NOTE: An illness or recovery is considered temporary if it will last 14 days or less.

d. If a resident is bedridden, how long is bedridden status expected to persist?

1. _____ (number of days)

2. _____ (estimated date illness or recovery is expected to end or when resident will no longer be confined to bed)

3. If illness or recovery is permanent, please explain: _____

e. Is resident receiving hospice care?

☐ No ☐ Yes If yes, specify the terminal illness: _____

18. PHYSICAL HEALTH STATUS: ☐ Good ☐ Fair ☐ Poor

19. COMMENTS:

20. PHYSICIAN'S NAME AND ADDRESS (PRINT)

21. TELEPHONE
()

22. LENGTH OF TIME RESIDENT HAS BEEN YOUR PATIENT

23. PHYSICIAN'S SIGNATURE

24. DATE

PREPLACEMENT APPRAISAL INFORMATION
Admission - Residential Care Facilities

NOTE: This information may be obtained from the applicant, or his/her authorized representative. (Relatives, social agency, hospital or physician may assist the applicant in completing this form.) This form is not a substitute for the Physician's Report (LIC 602).

APPLICANT'S NAME

AGE

HEALTH (Describe overall health condition including any dietary limitations)**PHYSICAL DISABILITIES** (Describe any physical limitations including vision, hearing or speech)**MENTAL CONDITION** (Specify extent of any symptoms of confusion, forgetfulness; participation in social activities (i.e., active or withdrawn))**HEALTH HISTORY** (List currently prescribed medications and major illnesses, surgery, accidents; specify whether hospitalized and length of hospitalization in last 5 years)**SOCIAL FACTORS** (Describe likes and dislikes, interests and activities)**BED STATUS**

- ☐ OUT OF BED ALL DAY
☐ IN BED ALL OR MOST OF THE TIME
☐ IN BED PART OF THE TIME

COMMENT:

TUBERCULOSIS INFORMATION

ANY HISTORY OF TUBERCULOSIS IN APPLICANT'S FAMILY?

- ☐ YES ☐ NO

DATE OF TB TEST

- ☐ POSITIVE
☐ NEGATIVE

ANY RECENT EXPOSURE TO ANYONE WITH TUBERCULOSIS?

- ☐ YES ☐ NO

ACTION TAKEN (IF POSITIVE)

GIVE DETAILS

AMBULATORY STATUS (this person is ☐ ambulatory ☐ nonambulatory)

Ambulatory means able to demonstrate the mental and physical ability to leave a building without the assistance of a person or the use of a mechanical device. An ambulatory person must be able to do the following:

- | YES | NO | |
|--------------------------|--------------------------|---|
| ☐ | ☐ | Able to walk without any physical assistance (e.g., walker, crutches, other person), or able to walk with a cane. |
| ☐ | ☐ | Mentally and physically able to follow signals and instructions for evacuation. |
| ☐ | ☐ | Able to use evacuation routes including stairs if necessary. |
| ☐ | ☐ | Able to evacuate reasonably quickly (e.g., walk directly the route without hesitation). |

FUNCTIONAL CAPABILITIES (Check all items below)

- | YES | NO | |
|--------------------------|--------------------------|--|
| ☐ | ☐ | Active, requires no personal help of any kind - able to go up and down stairs easily |
| ☐ | ☐ | Active, but has difficulty climbing or descending stairs |
| ☐ | ☐ | Uses brace or crutch |
| ☐ | ☐ | Feeble or slow |
| ☐ | ☐ | Uses walker. If Yes, can get in and out unassisted? ☐ Yes ☐ No |
| ☐ | ☐ | Uses wheelchair. If Yes, can get in and out unassisted? ☐ Yes ☐ No |
| ☐ | ☐ | Requires grab bars in bathroom |
| ☐ | ☐ | Other: (Describe) _____ |

SERVICES NEEDED (Check items and explain)

- | YES | NO | |
|--------------------------|--------------------------|--|
| ☐ | ☐ | Help in transferring in and out of bed and dressing _____ |
| ☐ | ☐ | Help with bathing, hair care, personal hygiene _____ |
| ☐ | ☐ | Does client desire and is client capable of doing own personal laundry and other household tasks (specify) _____ |
| ☐ | ☐ | Help with moving about the facility _____ |
| ☐ | ☐ | Help with eating (need for adaptive devices or assistance from another person) _____ |
| ☐ | ☐ | Special diet/observation of food intake _____ |
| ☐ | ☐ | Toileting, including assistance equipment, or assistance of another person _____ |
| ☐ | ☐ | Continence, bowel or bladder control. Are assistive devices such as a catheter required? _____ |
| ☐ | ☐ | Help with medication _____ |
| ☐ | ☐ | Needs special observation/night supervision (due to confusion, forgetfulness, wandering) _____ |
| ☐ | ☐ | Help in managing own cash resources _____ |
| ☐ | ☐ | Help in participating in activity programs _____ |
| ☐ | ☐ | Special medical attention _____ |
| ☐ | ☐ | Assistance in incidental health and medical care _____ |
| ☐ | ☐ | Other "Services Needed" not identified above _____ |

Is there any additional information which would assist the facility in determining applicant's suitability for admission? ☐ Yes ☐ No

If Yes, please attach comments on separate sheet.

To the best of my knowledge; I (the above person) do not need skilled nursing care.

SIGNATURE	DATE COMPLETED
APPLICANT (CLIENT) OR AUTHORIZED REPRESENTATIVE	
SIGNATURE	DATE COMPLETED
LICENSEE OR DESIGNATED REPRESENTATIVE	
SIGNATURE	DATE COMPLETED

**RELEASE OF CLIENT/RESIDENT
MEDICAL INFORMATION**

To: _____
(PHYSICIAN, CLINIC, HOSPITAL, HOSPICE, HOME HEALTH AGENCY, ATTENDING NURSE, PSYCHOLOGIST, COUNSELOR, THERAPIST, ETC.)

Date: _____

(ADDRESS)

I hereby authorize you to release any and all medical or confidential information contained in the record of:

(NAME OF PERSON)

(NAME AND ADDRESS OF FACILITY, PERSON OR AGENCY REQUESTING INFORMATION)

THIS AUTHORIZATION SHALL EXPIRE ON:

(DATE)

(CLIENT OR AUTHORIZED REPRESENTATIVE)

(RELATIONSHIP TO PERSON ON WHOM INFORMATION IS REQUESTED)

(ADDRESS)

- NOTE:**
1. The person who authorized this release may revoke this authorization at any time.
 2. The person who authorized this release has a right to receive a copy of the release.
 3. This information is required to conform to CCR Title 22 regulations, to ensure a continuum of care to the resident, client or child. Licensees should maintain a copy of this form in the facility records.
 4. The above facility is licensed by the Department of Social Services (or its accredited agencies), and does not provide skilled nursing care.

PERSONAL RIGHTS RESIDENTIAL CARE FACILITIES FOR THE ELDERLY

EXPLANATION: The California Code of Regulations, Title 22 requires that any person admitted to a facility must be advised of his/her personal rights. Facilities licensed for seven (7) or more are also required to post these rights in areas accessible to the public. Consequently, this form is designed to meet both the needs of persons admitted to facilities and the facility owners who are required to post these rights.

This form describes the personal rights to be afforded each person admitted to a facility. This form also provides the complaint procedures for the resident and the resident's responsible person. The facility staff, resident's responsible person or conservator must explain these rights in a manner appropriate to the resident's ability.

This form is to be reviewed, completed and signed by each resident, and/or responsible person (if any), or conservator upon admission to the facility. The resident and/or responsible person or conservator also has the right to receive a completed copy of the originally signed form. This originally signed copy shall be retained in the resident's file, which is maintained by the facility.

RESIDENT OR CONSERVATOR AND RESPONSIBLE PERSON

Upon satisfactory and full disclosure of the personal rights, complete the following:

I/we have been personally advised and have received a copy of the personal rights contained in the California Code of Regulations, Title 22.

(PRINT THE NAME OF THE FACILITY)

(PRINT THE ADDRESS OF THE FACILITY)

(PRINT THE NAME OF THE RESIDENT)

(SIGNATURE OF THE RESIDENT)

(DATE)

(SIGNATURE OF THE RESPONSIBLE PERSON OR CONSERVATOR)

(TITLE OF THE RESPONSIBLE PERSON OR CONSERVATOR)

THE RESIDENT AND/OR THE RESPONSIBLE PERSON OR CONSERVATOR HAS THE RIGHT TO BE INFORMED OF THE APPROPRIATE LICENSING AGENCY TO CONTACT REGARDING COMPLAINTS, WHICH IS:

NAME

ADDRESS

CITY

ZIP CODE

AREA CODE/TELEPHONE NUMBER

()

To report known or suspected elder abuse, contact the Statewide Ombudsman Toll Free 24-hour CRISIS line at 1-800-231-4024. Local Ombudsman's Office Telephone Number _____

PERSONAL RIGHTS

RESIDENTIAL CARE FACILITIES FOR THE ELDERLY

Explanation: Each resident shall have rights which include, but are not limited to, the following:

- (1) To be accorded dignity in his/her personal relationships with staff, residents, and other persons.
- (2) To be accorded safe, healthful and comfortable accommodations, furnishings and equipment.
- (3) To be free from corporal or unusual punishment, humiliation, intimidation, mental abuse, or other actions of a punitive nature, such as withholding of monetary allowances or interfering with daily living functions such as eating or sleeping patterns or elimination.
- (4) To be informed by the licensee of the provisions of law regarding complaints and of procedures to confidentially register complaints, including, but not limited to, the address and telephone number of the complaint receiving unit of the licensing agency.
- (5) To have the freedom of attending religious services or activities of his/her choice and to have visits from the spiritual advisor of his/her choice. Attendance at religious services, either in or outside the facility, shall be on a completely voluntary basis.
- (6) To leave or depart the facility at any time and to not be locked into any room, building, or on facility premises by day or night. This does not prohibit the establishment of house rules, such as the locking of doors at night, for the protection of residents; nor does it prohibit, with permission of the licensing agency, the barring of windows against intruders.
- (7) To visit the facility prior to residence along with his/her family and responsible persons.
- (8) To have his/her family or responsible persons regularly informed by the facility of activities related to his/her care or services including ongoing evaluations, as appropriate to the resident's needs.
- (9) To have communications to the facility from his/her family and responsible persons answered promptly and appropriately.

- (10) To be informed of the facility's policy concerning family visits and other communications with residents. This policy shall encourage regular family involvement and provide ample opportunities for family participation in activities at the facility.
- (11) To have his/her visitors, including ombudspersons and advocacy representatives permitted to visit privately during reasonable hours and without prior notice, provided that the rights of other residents are not infringed upon.
- (12) To wear his/her own clothes; to keep and use his/her own personal possessions, including his/her toilet articles; and to keep and be allowed to spend his/her own money.
- (13) To have access to individual storage space for private use.
- (14) To have reasonable access to telephones, to both make and receive confidential calls. The licensee may require reimbursement for long distance calls.
- (15) To mail and receive unopened correspondence in a prompt manner.
- (16) To receive or reject medical care, or other services.
- (17) To receive assistance in exercising the right to vote.
- (18) To move from the facility.

Reference: California Code of Regulations - Title 22, Section 87572, Residential Care Facilities for the Elderly

CLIENT/RESIDENT PERSONAL PROPERTY AND VALUABLES

Facilities must safeguard client's/resident's personal property/valuables entrusted to the facility. Licensee/Administrator is responsible for maintaining a record of personal property/valuables entrusted to and removed from the facility. Under "Number", enter the quantity of items entrusted. Under "Description", describe the item (marking articles by names or numbers may aid identification.). Under "Location", enter where items are stored. Licensee/Administrator and client/resident must sign each entry. Explain why, if client/resident does not sign. Provide a copy to the client/resident and maintain a copy in client's/resident's file. As property/valuable is removed, explain the reason for removal, enter the removal date, and ensure form is signed by all required persons specified above.

The reverse side of this form may be completed and retained in Residential Care Facilities for the Elderly to meet the notice requirements of Health and Safety Code Section 1569.153(k).

Name of Client/Resident	Social Security No.
-------------------------	---------------------

A. PERSONAL PROPERTY/VALUABLES ENTRUSTED TO FACILITY

[illegible]

B. PERSONAL PROPERTY/VALUABLES REMOVED

[illegible]

RESIDENTIAL CARE FACILITIES FOR THE ELDERLY ONLY.

Section 1569.152 of the Health and Safety Code:

1569.152. (a) A residential care facility for the elderly, as defined in Section 1569.2, which fails to make reasonable efforts to safeguard resident property shall reimburse a resident for or replace stolen or lost resident property at its then current value. The facility shall be presumed to have made reasonable efforts to safeguard resident property if the facility has shown clear and convincing evidence of its efforts to meet each of the requirements specified in Section 1569.153. The presumption shall be a rebuttable presumption, and the resident or the resident's representative may pursue this matter in any court of competent jurisdiction.

(b) A civil penalty shall be levied if the residential care facility for the elderly has no program in place or if the facility has not shown clear and convincing evidence of its efforts to meet all of the requirements set forth in Section 1569.153. The State Department of Social Services shall issue a deficiency in the event that the manner in which the policies have been implemented is inadequate or the individual facility situation warrants additional theft and loss protections.

(c) The department shall not determine that a facility's program is inadequate based solely on the occasional occurrence of theft or loss in a facility.

SEC. 3. Section 1569.153 of the Health and Safety Code:

1569.153. A theft and loss program shall be implemented by the residential care facilities for the elderly within 90 days after January 1, 1989. The program shall include all of the following:

(a) Establishment and posting of the facility's policy regarding theft and investigative procedures.

(b) Orientation to the policies and procedures for all employees within 90 days of employment.

(c) Documentation of lost and stolen resident property with a value of twenty-five dollars (\$25) or more within 72 hours of the discovery of the loss or theft and, upon request, the documented theft and loss record for the past 12 months shall be made available to the State Department of Social Services, law enforcement agencies and to the office of the State Long-Term Care Ombudsman in response to a specific complaint. The documentation shall include, but not be limited to, the following:

(1) A description of the article.

(2) Its estimated value.

(3) The date and time the theft or loss was discovered.

(4) If determinable, the date and time the loss or theft occurred.

(5) The action taken.

(d) A written resident personal property inventory is established upon admission and retained during the resident's stay in the residential care facility for the elderly. Inventories shall be written in ink, witnessed by the facility and the resident or resident's representative, and dated. A copy of the written inventory shall be provided to the resident or the person acting on the resident's behalf. All additions to an inventory shall be made in ink, and shall be witnessed by the facility and the resident or resident's representative, and dated. Subsequent items brought into or removed from the facility shall be added to or deleted from the personal property inventory by the facility at the written request of the resident, the resident's family, a responsible party, or a person acting on behalf of a resident. The facility shall not be liable for items which have not been requested to be included in the inventory or for items which have been deleted from the inventory. A copy of a current inventory shall be made available upon request to the resident, responsible party, or other authorized representative. The resident, resident's family, or a responsible party may list those items which are not subject to addition or deletion from the inventory such as personal clothing or laundry, which are subject to frequent removal from the facility.

(e) Inventory and surrender of the resident's personal effects and valuables upon discharge to the resident or authorized representative in exchange for a signed receipt.

(f) Inventory and surrender of personal effects and valuables following the death of a resident to the authorized representative in exchange for a signed receipt. Immediate written notice to the public administrator of the county upon the death of a resident whose heirs are unable or unwilling to claim the property as specified in Chapter 20 (commencing with Section 1140) of Division 3 of the Probate Code.

(g) Documentation, at least semiannually, of the facility's efforts to control theft and loss, including the review of theft and loss documentation and investigative procedures and results of the investigation by the administrator and, when feasible, the resident council.

(h) Establishment of a method of marking, to the extent feasible, personal property items for identification purposes upon admission and, as added to the property inventory list, including engraving of dentures and tagging of other prosthetic devices.

(i) Reports to the local law enforcement agency within 36 hours when the administrator of the facility has reason to believe resident property with a then current value of one hundred dollars (\$100) or more has been stolen. Copies of those reports for the preceding 12 months shall be made available to the State Department of Social Services and law enforcement agencies.

(j) Maintenance of a secured area for residents' property which is available for safekeeping of resident property upon the request of the resident or the resident's responsible party. Provide a lock for the resident's bedside drawer or cabinet upon request of and at the expense of the resident, the resident's family, or authorized representative. The facility administrator shall have access to the locked areas upon request.

(k) A copy of this section and Sections 1569.152, 1569.153 and 1569.154 is provided by a facility to all of the residents and their responsible parties, and available upon request, to all of the facility's prospective residents and their responsible parties.

(l) Notification to all current residents and all new residents, upon admission, of the facility's policies and procedures relating to the facility's theft and loss prevention program.

(m) Only those residential units in which there are no unrelated residents and where the unit can be secured by the resident or residents are exempt from the requirements of this section.

Section 1569.154 of the Health and Safety Code:

1569.154. No provision of a contract of admission, which includes all documents which a resident or his or her representative is required to sign at the time of, or as a condition of, admission to a residential care facility for the elderly, shall require or imply a lesser standard of responsibility for the personal property of residents than is required by law.

I have received a copy of Health and Safety Code Sections: 1569.152, 1569.153, and 1569.154 and am acquainted with the facilities personal property safeguard procedures.

CLIENT/RESIDENT/RESPONSIBLE PERSON SIGNATURE	TITLE	DATE
LICENSEE OR DESIGNATED REPRESENTATIVE		DATE COMPLETED

CENTRALLY STORED MEDICATION AND DESTRUCTION RECORD

I. CENTRALLY STORED MEDICATION

INSTRUCTIONS: Centrally stored medications shall be kept in a safe and locked place that is not accessible to any person(s) except authorized individuals. Medication records on each client/resident shall be maintained for at least one year.

[illegible]

**UNUSUAL INCIDENT/INJURY
REPORT****INSTRUCTIONS :** NOTIFY LICENSING AGENCY, PLACEMENT AGENCY AND
RESPONSIBLE PERSONS, IF ANY, BY NEXT WORKING DAY.

SUBMIT WRITTEN REPORT WITHIN 7 DAYS OF OCCURRENCE.

RETAIN COPY OF REPORT IN CLIENT'S FILE.

NAME OF FACILITY	FACILITY FILE NUMBER	TELEPHONE NUMBER ()
ADDRESS	CITY, STATE, ZIP	

CLIENTS/RESIDENTS INVOLVED	DATE OCCURRED	AGE	SEX	DATE OF ADMISSION

TYPE OF INCIDENT

- | | | | | |
|--|---|--|---|---|
| ☐ Unauthorized Absence | ☐ Alleged Client Abuse | ☐ Rape | ☐ Injury-Accident | ☐ Medical Emergency |
| ☐ Aggressive Act/Self | ☐ Sexual | ☐ Pregnancy | ☐ Injury-Unknown Origin | ☐ Other Sexual Incident |
| ☐ Aggressive Act/Another Client | ☐ Physical | ☐ Suicide Attempt | ☐ Injury-From another Client | ☐ Theft |
| ☐ Aggressive Act/Staff | ☐ Psychological | ☐ Other | ☐ Injury-From behavior episode | ☐ Fire |
| ☐ Aggressive Act/Family, Visitors | ☐ Financial | | ☐ Epidemic Outbreak | ☐ Property Damage |
| ☐ Alleged Violation of Rights | ☐ Neglect | | ☐ Hospitalization | ☐ Other (<i>explain</i>) |

DESCRIBE EVENT OR INCIDENT (INCLUDE DATE, TIME, LOCATION, PERPETRATOR, NATURE OF INCIDENT, ANY ANTECEDENTS LEADING UP TO INCIDENT AND HOW CLIENTS WERE AFFECTED, INCLUDING INJURIES:

PERSON(S) WHO OBSERVED THE INCIDENT/INJURY:

EXPLAIN WHAT IMMEDIATE ACTION WAS TAKEN (INCLUDE PERSONS CONTACTED):

MEDICAL TREATMENT NECESSARY? ☐ YES ☐ NO IF YES, GIVE NATURE OF TREATMENT:

WHERE ADMINISTERED:

ADMINISTERED BY:

FOLLOW-UP TREATMENT, IF ANY:

ACTION TAKEN OR PLANNED (BY WHOM AND ANTICIPATED RESULTS:

LICENSEE/SUPERVISOR COMMENTS:

NAME OF ATTENDING PHYSICIAN

REPORT SUBMITTED BY:

NAME AND TITLE

DATE

REPORT REVIEWED/APPROVED BY:

NAME AND TITLE

DATE

AGENCIES/INDIVIDUALS NOTIFIED (SPECIFY NAME AND TELEPHONE NUMBER)

- ☐ LICENSING ☐ ADULT/CHILD PROTECTIVE SERVICES
- ☐ LONG TERM CARE OMBUDSMAN ☐ PARENT/GUARDIAN/CONSERVATOR
- ☐ LAW ENFORCEMENT ☐ PLACEMENT AGENCY

DEATH REPORT

ENSEE MUST REPORT THE DEATH OF A CLIENT OF ANY CAUSE, REGARDLESS OF WHERE THE DEATH OCCURRED.

INSTRUCTIONS : NOTIFY LICENSING AGENCY, PLACEMENT AGENCY AND RESPONSIBLE PERSONS, IF ANY, BY NEXT WORKING DAY.
SUBMIT WRITTEN REPORT WITHIN 7 DAYS OF OCCURRENCE.
RETAIN COPY OF REPORT IN CLIENT'S FILE.

NAME OF FACILITY	FACILITY FILE NUMBER	TELEPHONE NUMBER ()	
ADDRESS	CITY, STATE, ZIP		
CLIENT'S NAME	D.O.B.	SEX	DATE OF ADMISSION
DATE AND TIME OF DEATH	PLACE OF DEATH		

DESCRIBE IMMEDIATE CAUSE OF DEATH (IF CORONER REPORT MADE, SEND COPY WITHIN 30 DAYS):

DESCRIBE CONDITIONS PRIOR TO OR CONTRIBUTING TO DEATH:

EXPLAIN WHAT IMMEDIATE ACTION WAS TAKEN (INCLUDE PERSONS CONTACTED):

MEDICAL TREATMENT NECESSARY? ☐ YES ☐ NO IF YES, GIVE NATURE OF TREATMENT:

--

NAME OF ATTENDING PHYSICIAN		NAME OF MORTICIAN	
REPORT SUBMITTED BY:	NAME AND TITLE	DATE	
REPORT REVIEWED/APPROVED BY:	NAME AND TITLE	DATE	

AGENCIES/INDIVIDUALS NOTIFIED (SPECIFY NAME AND TELEPHONE NUMBER)

☐ LICENSING _____	☐ ADULT/CHILD PROTECTIVE SERVICES _____
☐ LONG TERM CARE OMBUDSMAN _____	☐ PARENT/GUARDIAN/CONSERVATOR _____
☐ LAW ENFORCEMENT _____	☐ PLACEMENT AGENCY _____

**CONSENT FOR EMERGENCY MEDICAL TREATMENT-
Adult and Elderly Residential Facilities**

AS THE CLIENT, AUTHORIZED REPRESENTATIVE OR CONSERVATOR, I HEREBY GIVE CONSENT TO

_____ TO PROVIDE ALL EMERGENCY MEDICAL OR DENTAL CARE
FACILITY NAME
PRESCRIBED BY A DULY LICENSED PHYSICIAN (M.D.) OSTEOPATH (D.O.) OR DENTIST (D.D.S.) FOR

_____. THIS CARE MAY BE GIVEN UNDER WHATEVER
NAME
CONDITIONS ARE NECESSARY TO PRESERVE THE LIFE, LIMB OR WELL BEING OF THE INDIVIDUAL NAMED
ABOVE.

CLIENT HAS THE FOLLOWING MEDICATION ALLERGIES:

DATE

CLIENT/AUTHORIZED REPRESENTATIVE/CONSERVATOR SIGNATURE
(CIRCLE APPROPRIATE TITLE)

HOME ADDRESS

HOME PHONE

()

WORK PHONE

()

ORIGINAL - Client/Client Representative**COPY - Client/Resident File****TELECOMMUNICATIONS DEVICE NOTIFICATION**

- | | |
|---|---|
| ☐ ADULT RESIDENTIAL FACILITY | ☐ FOSTER FAMILY HOME |
| ☐ ADULT DAY PROGRAM | ☐ GROUP HOME |
| ☐ RESIDENTIAL CARE FACILITY FOR THE CHRONICALLY ILL | ☐ SMALL FAMILY HOME |
| ☐ SOCIAL REHABILITATION FACILITY | ☐ RESIDENTIAL CARE FACILITY FOR THE ELDERLY |

NOTICE

Any deaf or hearing impaired, or otherwise impaired resident of any community care facility is entitled to equipment and service, pursuant to Section 2881 of the California Public Utilities Code, to improve the quality of their telecommunications. Any resident who has a declaration from a licensed professional or a qualified state or federal agency, that he or she is deaf or hearing impaired, or otherwise disabled should contact the California Telephone Access Program at 1-800-806-1191 and ask for assistance in obtaining this equipment and service.

This section shall not be construed to require, in any way, the licensee to provide a separate telephone line for any resident.

CLIENT/RESIDENT SIGNATURE		DATE
CONSERVATOR/RESPONSIBLE PERSON/AUTHORIZED REPRESENTATIVE SIGNATURE (IF ANY)		DATE
FACILITY NAME	FACILITY ADDRESS	
FACILITY REPRESENTATIVE SIGNATURE		DATE

**CALIFORNIA PUBLIC UTILITIES CODE
SECTION 2881 (a) and (c)**

2881. (a) The commission shall design and implement a program to provide a telecommunications device capable of serving the needs of individuals who are deaf or hearing impaired, together with a single party line, at no charge additional to the basic exchange rate, to any subscriber who is certified as an individual who is deaf or hearing impaired by a licensed physician and surgeon, audiologist, or a qualified state or federal agency, as determined by the commission, and to any subscriber that is an organization representing individuals who are deaf or hearing impaired, as determined and specified by the commission pursuant to subdivision (e). A licensed hearing aid dispenser may certify the need of an individual to participate in the program if that individual has been previously fitted with an amplified device by the dispenser and the dispenser has the individual's hearing records on file prior to certification.

(c) The commission shall also design and implement a program whereby specialized or supplemental telephone communications equipment may be provided to subscribers who are certified to be disabled at no charge additional to the basic exchange rate. The certification, including a statement of visual or medical need for specialized telecommunications equipment, shall be provided by a licensed optometrist or physician and surgeon acting within the scope of practice of his or her license, or by a qualified state or federal agency as determined by the commission.



CA PRN Authorization Letter

Please FAX back to: {Insert Community Name} FAX #:

Dear Dr.

Regarding Your patient:

To receive nonprescription PRN medications, state licensing requires that either:

- 1) your patient be capable of determining his/her own need for medication, or
- 2) for non-prescription medication only, be able to clearly communicate his/her symptoms.

If your patient cannot determine his/her need for a medication, or, clearly communicate the symptoms for a nonprescription medication then you, the physician, must be contacted before the PRN medication can be given. Your completion of this form will serve to document your patient's current ability to determine his/her own need for these medications.

As a licensed care provider, it is my responsibility to monitor your patient's continued ability to determine his/her own need for PRN medications and inform you of any changes which indicate he/she can no longer make these decisions.

Thank you for your assistance.

Sincerely,

Signature and Title: _____

Telephone number: _____ Date: _____

Please check which circumstance describes your patient and fax back to the Community. Thank you.

☐ My patient can determine and clearly communicate his/her need for prescription and nonprescription medication on a PRN basis.

☐ My patient cannot determine his/her own need for nonprescription PRN medication but can clearly communicate his/her symptoms indicating a need for a nonprescription medication.

☐ My patient cannot determine his/her need for prescription and/or nonprescription PRN medication and cannot communicate his/her symptoms indicating a need for a nonprescription medication. (Requires contact with Physician prior to each dose)

The following prescription and nonprescription medications can be taken on a PRN basis:

Physician's Signature: _____ Date: _____

Rights of Resident Councils

This is posted in accordance with Health and Safety Code section 1569.157

(a) Every licensed residential care facility for the elderly, at the request of two or more residents, shall assist the residents in establishing and maintaining a single resident council at the facility. The resident council shall be composed of residents of the facility. Family members, resident representatives, advocates, long-term care ombudsman program representatives, facility staff, or others may participate in resident council meetings and activities at the invitation of the resident council.

(b) A resident council may, among other things, make recommendations to facility administrators to improve the quality of daily living and care in the facility and to promote and protect residents' rights.

(c) If a resident council submits written concerns or recommendations, the facility shall respond in writing regarding any action or inaction taken in response to those concerns or recommendations within 14 calendar days.

(d) Facility policies on resident councils shall not limit the right of residents to meet independently with outside persons or facility personnel.

(e) Each resident council member shall be informed by the facility of his or her right to be interviewed as part of the regulatory inspection process.

(f) Facilities shall promote resident councils as follows:

(1) If a facility has a resident council, the facility shall inform new residents of the existence of the resident council. The facility shall also provide information on the time, place, and dates of resident council meetings and the resident representative to contact regarding involvement in the resident council.

(2) If a facility has a resident council and a licensed capacity of 16 or more, the facility shall appoint a designated staff liaison to assist the resident council, make a room available for resident council meetings, and post meeting information in a central location readily accessible to residents, relatives, and resident representatives.

(3) If a facility does not have a resident council, upon admission, the facility shall provide written information on the resident's right to form a resident council to the resident and the resident representative, as indicated in the admissions agreement.

(4) Upon request, and with the permission of the resident council, the facility shall share the name and contact information of the designated representative of the resident council with the long-term care ombudsman program.

(g) A facility shall not willfully interfere with the formation, maintenance, or promotion of a resident council, or its participation in the regulatory inspection process. For the purposes of this subdivision, willful interference shall include, but not be limited to, discrimination or retaliation in any way against an individual as a result of his or her participation in a resident council, refusal to publicize resident council meetings or provide appropriate space for either meetings or a bulletin board, or failure to respond to written requests by the resident council in a timely manner.

(h) The text of this section with the heading "Rights of Resident Councils" shall be posted in a prominent place at the facility accessible to residents, family members, and resident representatives.

(i) A violation of this section shall not be subject to the provisions of Section 1569.40. A violation of this section shall constitute a violation of resident rights. A facility that violates this section shall be subject to a daily civil penalty of two hundred fifty dollars (\$250) until the violation is corrected. A violation shall be deemed to have been corrected on the date the facility submits documentation of the correction to the department if the correction is verified by the department.



CA Reporting Requirements

..... Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Each licensee (hereafter referred to as the Community) will furnish to the RCFE licensing agency any reports the Department requires. See also *Suspected Abuse* policy.

Guidelines

Immediate Report Requirements

Fires or explosions which occur in or on the premises will be reported immediately to the local fire authority; in areas not having organized fire services, within 24 hours to the State Fire Marshal; and no later than the next working day to the licensing agency.

Twenty-Four Hour Phone/Fax Report Requirements

Occurrences, such as epidemic outbreaks, poisonings, catastrophes or major accidents which threaten the welfare, safety or health of residents, personnel or visitors, will be reported within 24 hours either by telephone or facsimile to the licensing agency and to the local health officer when appropriate.

Seven Day Written Report Requirements

The Community will submit a written report to the local licensing agency and to the person responsible for the resident within seven days of the occurrence of any of the events specified in A through D below. This report will include the residents name, age, sex and date of admission; date and nature of event; attending physician's name, findings, and treatment if any; and disposition of the case.

- A. Death of any resident from any cause regardless of where the death occurred, including but not limited to a day program, a hospital, enroute to or from a hospital, or visiting away from the Community.
- B. Any serious injury as determined by the attending physician and occurring while the resident is under Community supervision.
- C. The use of and Automated External Defibrillator (AED).
- D. Any incident which threatens the welfare, safety or health of any resident, such as physical or psychological abuse of a resident by team members or other residents, or unexplained absence of any resident.

Thirty Day Report Requirements

The community will notify the state licensing agency, in writing, within 30 days of the hiring of a new administrator (General Manager). The notification will include the following:

- Name, residence and mailing addresses of the new administrator.
- Date he/she assumed the position.

- Description of his/her background and qualifications, including documentation of required education and administrator certification. (photocopies of the documentation are acceptable).

CONFIDENTIAL REPORT - NOT SUBJECT TO PUBLIC DISCLOSURE

REPORT OF SUSPECTED DEPENDENT ADULT/ELDER ABUSE

DATE COMPLETED _____

TO BE COMPLETED BY REPORTING PARTY. PLEASE PRINT OR TYPE. SEE GENERAL INSTRUCTIONS.

A. VICTIM ☐ Check box if victim consents to disclosure of information (Ombudsman use only - WIC 15636(a))

NAME (LAST NAME, FIRST NAME) _____

AGE _____

DATE OF BIRTH _____

SSN _____

GENDER

☐ M ☐ F

ETHNICITY _____

LANGUAGE (✓ CHECK ONE)

☐ NON-VERBAL☐ ENGLISH☐ OTHER (SPECIFY) _____

ADDRESS (IF FACILITY, INCLUDE NAME AND NOTIFY OMBUDSMAN) _____

CITY _____

ZIP CODE _____

TELEPHONE _____

PRESENT LOCATION (IF DIFFERENT FROM ABOVE) _____

CITY _____

ZIP CODE _____

TELEPHONE _____

☐ ELDERLY (65+)☐ DEVELOPMENTALLY DISABLED☐ MENTALLY ILL/DISABLED☐ LIVES ALONE☐ PHYSICALLY DISABLED☐ UNKNOWN/OTHER☐ LIVES WITH OTHERS

B. SUSPECTED ABUSER ✓ Check if ☐ Self-Neglect

NAME OF SUSPECTED ABUSER _____

ADDRESS _____

CITY _____

ZIP CODE _____

TELEPHONE _____

☐ CARE CUSTODIAN (type) _____☐ PARENT☐ SON/DAUGHTER☐ OTHER _____☐ HEALTH PRACTITIONER (type) _____☐ SPOUSE☐ OTHER RELATION _____

GENDER

☐ M ☐ F

ETHNICITY _____

AGE _____

D.O.B. _____

HEIGHT _____

WEIGHT _____

EYES _____

HAIR _____

C. REPORTING PARTY Check appropriate box if reporting party waives confidentiality to: ☐ All ☒ All but victim ☐ All but perpetrator

NAME _____

SIGNATURE _____

OCCUPATION _____

AGENCY/NAME OF BUSINESS _____

RELATION TO VICTIM/HOW ABUSE IS KNOWN _____

STREET _____

CITY _____

ZIP CODE _____

TELEPHONE _____

E-MAIL ADDRESS _____

D. INCIDENT INFORMATION - Address where incident occurred

DATE/TIME OF INCIDENT(S) _____

PLACE OF INCIDENT (✓ CHECK ONE)

☐ OWN HOME☐ COMMUNITY CARE FACILITY☐ HOSPITAL/ACUTE CARE HOSPITAL☐ HOME OF ANOTHER☐ NURSING FACILITY/SWING BED☐ OTHER (Specify) _____

E. REPORTED TYPES OF ABUSE (✓ CHECK ALL THAT APPLY)

1. PERPETRATED BY OTHERS (WIC 15610.07 & 15610.63)

a. ☐ PHYSICAL (e.g. assault/battery, constraint or deprivation, chemical restraint, over/under medication)b. ☐ SEXUALc. ☐ FINANCIALe. ☐ ABANDONMENTf. ☐ ISOLATIONd. ☐ NEGLECT (including Deprivation of Goods and Services by a Care Custodian)g. ☐ ABDUCTIONh. ☐ PSYCHOLOGICAL/MENTALi. ☐ OTHER _____

2. SELF-NEGLECT (WIC 15610.57(b)(5))

a. ☐ PHYSICAL CARE (e.g. personal hygiene, food, clothing, shelter)d. ☐ MALNUTRITION/DEHYDRATIONb. ☐ MEDICAL CARE (e.g. physical and mental health needs)e. ☐ FINANCIAL SELF-NEGLECT (e.g. inability to manage one's own personal finances)c. ☐ HEALTH and SAFETY HAZARDS (e.g. risk of suicide, unsafe environment)f. ☐ OTHER _____

ABUSE RESULTED IN (✓ CHECK ALL THAT APPLY)

☐ NO PHYSICAL INJURY ☐ MINOR MEDICAL CARE ☐ HOSPITALIZATION ☐ CARE PROVIDER REQUIRED☐ DEATH ☐ MENTAL SUFFERING ☐ SERIOUS BODILY INJURY* ☐ OTHER (SPECIFY) _____☐ UNKNOWN

F. REPORTER'S OBSERVATIONS, BELIEFS, AND STATEMENTS BY VICTIM IF AVAILABLE. DOES ALLEGED PERPETRATOR STILL HAVE ACCESS TO THE VICTIM? DOES THE ALLEGATION INVOLVE A SERIOUS BODILY INJURY (see definition in section "Reporting Responsibilities and Time Frames" within the General Instructions)? PROVIDE ANY KNOWN TIME FRAME (2 days, 1 week, ongoing, etc.). LIST ANY POTENTIAL DANGER FOR INVESTIGATOR (animals, weapons, communicable diseases, etc.).

☐ ☒ CHECK IF MEDICAL, FINANCIAL (ACCOUNT INFORMATION, ETC.), PHOTOGRAPHS, OR OTHER SUPPLEMENTAL INFORMATION IS ATTACHED.

G. OTHER PERSON BELIEVED TO HAVE KNOWLEDGE OF ABUSE (family, significant others, neighbors, medical providers, agencies involved, etc.)

NAME	RELATIONSHIP
ADDRESS	TELEPHONE ()

H. FAMILY MEMBER OR OTHER PERSON RESPONSIBLE FOR VICTIM'S CARE (If unknown, list contact person)

NAME	IF CONTACT PERSON ONLY ☒ CHECK ☐			RELATIONSHIP
ADDRESS	CITY	ZIP CODE	TELEPHONE ()	

I. TELEPHONE REPORT MADE TO ☐ APS ☐ Law Enforcement ☐ Local Ombudsman ☐ Calif. Dept. of State Hospitals
☐ Calif. Dept. of Developmental Services

NAME OF OFFICIAL CONTACTED BY PHONE	TELEPHONE ()	DATE/TIME
-------------------------------------	------------------	-----------

J. WRITTEN REPORT Enter information about the agencies receiving this report. If the abuse occurred in a LTC facility and resulted in Serious Bodily Injury*, please refer to "Reporting Responsibilities and Time Frames" in the General Instructions. Do not submit report to California Department of Social Services Adult Programs Division.

AGENCY NAME	ADDRESS OR FAX	☐ Date Mailed	☐ Date Faxed
AGENCY NAME	ADDRESS OR FAX	☐ Date Mailed	☐ Date Faxed
AGENCY NAME	ADDRESS OR FAX	☐ Date Mailed	☐ Date Faxed

K. RECEIVING AGENCY USE ONLY ☐ Telephone Report ☐ Written Report

1. Report Received by	Date/Time
-----------------------	-----------

2. Assigned ☐ Immediate Response ☐ Ten-Day Response ☐ No Initial Response (NIR)
☐ Not APS ☐ Not Ombudsman ☐ No Ten-Day (NTD)

Approved by Assigned to (optional)

3. Cross-Reported to ☐ CDPH-Licensing & Cert.; ☐ CDSS-CCL; ☐ Local Ombudsman; ☐ Bureau of Medi-Cal Fraud & Elder Abuse;
☐ Calif. Dept. of State Hospitals; ☐ Law Enforcement; ☐ Professional Licensing Board;
☐ Calif. Dept. of Developmental Services; ☐ APS;
☐ Other (Specify) Date of Cross-Report

4. APS/Ombudsman/Law Enforcement Case File Number

REPORT OF SUSPECTED DEPENDENT ADULT/ELDER ABUSE GENERAL INSTRUCTIONS

PURPOSE OF FORM

This form, as adopted by the California Department of Social Services (CDSS), is required under Welfare and Institutions Code (WIC) Sections 15630 and 15658(a)(1). This form documents the information given by the reporting party on the suspected incident of abuse or neglect of an elder or dependent adult. **Abuse** means any treatment with resulting physical harm, pain, or mental suffering or the deprivation by a care custodian of goods or services that are necessary to avoid physical harm or mental suffering. **Neglect** means the negligent failure of an elder or dependent adult or of any person having the care or custody of an elder or a dependent adult to exercise that degree of self-care or care that a reasonable person in a like position would exercise. **Elder** means any person residing in this state who is 65 years of age or older (WIC Section 15610.27). **Dependent Adult** means any person residing in this state, between the ages of 18 and 64, who has physical or mental limitations that restrict his or her ability to carry out normal activities or to protect his or her rights including, but not limited to, persons who have physical or developmental disabilities or whose physical or mental abilities have diminished because of age (WIC Section 15610.23). Dependent adult includes any person between the ages of 18 and 64 who is admitted as an inpatient to a 24-hour health facility (defined in the Health and Safety Code Sections 1250, 1250.2, and 1250.3).

COMPLETION OF THE FORM

1. This form may be used by the receiving agency to record information through a telephone report of suspected dependent adult/elder abuse.
2. If any item of information is unknown, enter "unknown."
3. Item A: Check box to indicate if the victim waives confidentiality.
4. Item C: Check box if the reporting party waives confidentiality. Please note that mandated reporters are required to disclose their names, however, non-mandated reporters may report anonymously.

REPORTING RESPONSIBILITIES AND TIME FRAMES:

Any mandated reporter, who in his or her professional capacity, or within the scope of his or her employment, has observed or has knowledge of an incident that reasonably appears to be abuse or neglect, or is told by an elder or dependent adult that he or she has experienced behavior constituting abuse or neglect, or reasonably suspects that abuse or neglect has occurred, shall complete this form for each report of known or suspected instance of abuse (physical abuse, sexual abuse, financial abuse, abduction, neglect (self-neglect), isolation, and abandonment) involving an elder or dependent adult.

***Serious bodily injury** means an injury involving extreme physical pain, substantial risk of death, or protracted loss or impairment of function of a bodily member, organ or of mental faculty, or requiring medical intervention, including, but not limited to, hospitalization, surgery, or physical rehabilitation (WIC Section 15610.67).

Reporting shall be completed as follows:

- If the abuse occurred in a Long-Term Care (LTC) facility (as defined in WIC Section 15610.47) and resulted in serious bodily injury, report by telephone to the local law enforcement agency immediately and no later than two (2) hours after observing, obtaining knowledge of, or suspecting physical abuse. Send the written report to the local law enforcement agency, the local Long-Term Care Ombudsman Program (LTCOP), and the appropriate licensing agency (for long-term health care facilities, the California Department of Public Health; for community care facilities, the California Department of Social Services) within two (2) hours of observing, obtaining knowledge of, or suspecting physical abuse.
- If the abuse occurred in a LTC facility, was physical abuse, but did not result in serious bodily injury, report by telephone to the local law enforcement agency within 24 hours of observing, obtaining knowledge of, or suspecting physical abuse. Send the written report to the local law enforcement agency, the local LTCOP, and the appropriate licensing agency (for long-term health care facilities, the California Department of Public Health; for community care facilities, the California Department of Social Services) within 24 hours of observing, obtaining knowledge of, or suspecting physical abuse.
- If the abuse occurred in a LTC facility, was physical abuse, did not result in serious bodily injury, and was perpetrated by a resident with a physician's diagnosis of dementia, report by telephone to the local law enforcement agency or the local LTCOP, immediately or as soon as practicably possible. Follow by sending the written report to the LTCOP or the local law enforcement agency within 24 hours of observing, obtaining knowledge of, or suspecting physical abuse.
- If the abuse occurred in a LTC facility, was abuse other than physical abuse, report by telephone to the LTCOP or the law enforcement agency immediately or as soon as practicably possible. Follow by sending the written report to the local law enforcement agency or the LTCOP within two working days.

- If the abuse occurred in a state mental hospital or a state developmental center, mandated reporters shall report by telephone or through a confidential Internet reporting tool (established in WIC Section 15658) immediately or as soon as practicably possible and submit the report within two (2) working days of making the telephone report to the responsible agency as identified below:
 - If the abuse occurred in a State Mental Hospital, report to the local law enforcement agency or the California Department of State Hospitals.
 - If the abuse occurred in a State Developmental Center, report to the local law enforcement agency or to the California Department of Developmental Services.
- For all other abuse, mandated reporters shall report by telephone or through a confidential Internet reporting tool to the adult protective services agency or the local law enforcement agency immediately or as soon as practicably possible. If reported by telephone, a written or an Internet report shall be sent to adult protective services or law enforcement within two working days.

REPORTING PARTY DEFINITIONS

Mandated Reporter (WIC Section 15630 (a)) Any person who has assumed full or intermittent responsibility for care or custody of an elder or dependent adult, whether or not that person receives compensation, including administrators, supervisors, and any licensed staff of a public or private facility that provides care or services for elder or dependent adults, or any elder or dependent adult care custodian, health practitioner, clergy member, or employee of a county adult protective services agency or a local law enforcement agency, is a mandated reporter.

Care Custodian (WIC Section 15610.17) means an administrator or an employee of any of the following public or private facilities or agencies, or persons providing care or services for elders or dependent adults, including members of the support staff and maintenance staff: (a) Twenty-four hour health facilities, as defined in Sections 1250, 1250.2, and 1250.3 of the Health and Safety Code; (b) Clinics; (c) Home health agencies; (d) Agencies providing publicly funded in-home supportive services, nutrition services, or other home and community-based support services; (e) Adult day health care centers and adult day care; (f) Secondary schools that serve 18- to 22-year-old dependent adults and postsecondary educational institutions that serve dependent adults or elders; (g) Independent living centers; (h) Camps; (i) Alzheimer's Disease Day Care Resource Centers; (j) Community care facilities, as defined in Section 1502 of the Health and Safety Code, and residential care facilities for the elderly, as defined in Section 1569.2 of the Health and Safety Code; (k) Respite care facilities; (l) Foster homes; (m) Vocational rehabilitation facilities and work activity centers; (n) Designated area agencies on aging; (o) Regional centers for persons with developmental disabilities; (p) State Department of Social Services and State Department of Health Services licensing divisions; (q) County welfare departments; (r) Offices of patients' rights advocates and clients' rights advocates, including attorneys; (s) The Office of the State Long-Term Care Ombudsman; (t) Offices of public conservators, public guardians, and court investigators; (u) Any protection or advocacy agency or entity that is designated by the Governor to fulfill the requirements and assurances of the following: (1) The federal Developmental Disabilities Assistance and Bill of Rights Act of 2000, contained in Chapter 144 (commencing with Section 15001) of Title 42 of the United States Code, for protection and advocacy of the rights of persons with developmental disabilities; or (2) The Protection and Advocacy for the Mentally Ill Individuals Act of 1986, as amended, contained in Chapter 114 (commencing with Section 10801) of Title 42 of the United States Code, for the protection and advocacy of the rights of persons with mental illness; (v) Humane societies and animal control agencies; (w) Fire departments; (x) Offices of environmental health and building code enforcement; or (y) Any other protective, public, sectarian, mental health, or private assistance or advocacy agency or person providing health services or social services to elders or dependent adults.

Health Practitioner (WIC Section 15610.37) means a physician and surgeon, psychiatrist, psychologist, dentist, resident, intern, podiatrist, chiropractor, licensed nurse, dental hygienist, licensed clinical social worker or associate clinical social worker, marriage, family, and child counselor, or any other person who is currently licensed under Division 2 (commencing with Section 500) of the Business and Professions Code, any emergency medical technician I or II, paramedic, or person certified pursuant to Division 2.5 (commencing with Section 1797) of the Health and Safety Code, a psychological assistant registered pursuant to Section 2913 of the Business and Professions Code, a marriage, family, and child counselor trainee, as defined in subdivision (c) of Section 4980.03 of the Business and Professions Code, or an unlicensed marriage, family, and child counselor intern registered under Section 4980.44 of the Business and Professions Code, state or county public health or social service employee who treats an elder or a dependent adult for any condition, or a coroner.

Any officer and/or employee of a financial institution is a mandated reporter of suspected financial abuse and shall report suspected financial abuse of an elder or dependent adult on form SOC 342, "Report of Suspected Dependent Adult/Elder Financial Abuse".

MULTIPLE REPORTERS

When two or more mandated reporters are jointly knowledgeable of a suspected instance of abuse of a dependent adult or elder, and when there is agreement among them, the telephone report may be made by one member of the group. Also, a single written report may be completed by that member of the group. Any person of that group, who believes the report was not submitted, shall submit the report.

IDENTITY OF THE REPORTER

The identity of all persons who report under WIC Chapter 11 shall be confidential and disclosed only among APS agencies, local law enforcement agencies, LTCOPs, California State Attorney General Bureau of Medi-Cal Fraud and Elder Abuse, licensing agencies or their counsel, Department of Consumer Affairs Investigators (who investigate elder and dependent adult abuse), the county District Attorney, the Probate Court, and the Public Guardian. Confidentiality may be waived by the reporter or by court order.

FAILURE TO REPORT

Failure to report by mandated reporters (as defined under "Reporting Party Definitions") any suspected incidents of physical abuse (including sexual abuse), abandonment, isolation, financial abuse, abduction, or neglect (including self-neglect) of an elder or a dependent adult is a misdemeanor, punishable by not more than six months in the county jail, or by a fine of not more than \$1,000, or by both imprisonment and fine. Any mandated reporter who willfully fails to report abuse of an elder or a dependent adult, where the abuse results in death or great bodily injury, may be punished by up to one year in the county jail, or by a fine of up to \$5,000, or by both imprisonment and fine (WIC Section 15630(h)).

Officers or employees of financial institutions are mandated reporters of financial abuse (effective January 1, 2007). These mandated reporters who fail to report financial abuse of an elder or dependent adult are subject to a civil penalty not exceeding \$1,000. Individuals who willfully fail to report financial abuse of an elder or dependent adult are subject to a civil penalty not exceeding \$5,000. These civil penalties shall be paid by the financial institution, which is the employer of the mandated reporter, to the party bringing the action.

EXCEPTIONS TO REPORTING

Per WIC Section 15630(b)(3)(A), a mandated reporter who is a physician and surgeon, a registered nurse, or a psychotherapist, as defined in Section 1010 of the Evidence Code, shall not be required to report a suspected incident of abuse where all of the following conditions exist:

- (1) The mandated reporter has been told by an elder or a dependent adult that he or she has experienced behavior constituting physical abuse (including sexual abuse), abandonment, isolation, financial abuse, abduction, or neglect (including self-neglect).
- (2) The mandated reporter is not aware of any independent evidence that corroborates the statement that the abuse has occurred.
- (3) The elder or the dependent adult has been diagnosed with a mental illness or dementia, or is the subject of a court-ordered conservatorship because of a mental illness or dementia.
- (4) In the exercise of clinical judgment, the physician and surgeon, the registered nurse, or the psychotherapist, as defined in Section 1010 of the Evidence Code, reasonably believes that the abuse did not occur.

DISTRIBUTION OF SOC 341 COPIES

Mandated reporter: After making the telephone report to the appropriate agency or agencies, the reporter shall send the written report to the designated agencies (as defined under "Reporting Responsibilities and Time Frames"); and keep one copy for the reporter's file.

Receiving agency: Place the original copy in the case file. Send a copy to a cross-reporting agency, if applicable.

DO NOT SEND A COPY TO THE CALIFORNIA DEPARTMENT OF SOCIAL SERVICES ADULT PROGRAMS DIVISION.

**STATEMENT ACKNOWLEDGING REQUIREMENT TO REPORT
SUSPECTED ABUSE OF DEPENDENT ADULTS AND ELDERS****NOTE: RETAIN IN EMPLOYEE/ VOLUNTEER FILE**

NAME _____

POSITION _____

FACILITY _____

California law **REQUIRES** certain persons to report known or suspected abuse of dependent adults or elders. As an employee or volunteer at a licensed facility, you are one of those persons - a "mandated reporter."

PERSONS WHO ARE REQUIRED TO REPORT ABUSE

Mandated reporters include care custodians and any person who has assumed full or intermittent responsibility for care or custody of an elder or dependent adult, whether or not paid for that responsibility (Welfare and Institutions Code (WIC) Section 15630(a)). **Care custodian** means an administrator or an employee of most public or private facilities or agencies, or persons providing care or services for elders or dependent adults, including members of the support staff and maintenance staff (WIC Section 15610.17).

PERSONS WHO ARE THE SUBJECT OF THE REPORT

Elder means any person residing in this state who is 65 years of age or older (WIC Section 15610.27). **Dependent Adult** means any person residing in this state, between the ages of 18 and 64, who has physical or mental limitations that restrict his or her ability to carry out normal activities or to protect his or her rights including, but not limited to, persons who have physical or developmental disabilities or whose physical or mental abilities have diminished because of age and those admitted as inpatients in 24-hour health facilities (WIC Section 15610.23).

REPORTING RESPONSIBILITIES AND TIME FRAMES

Any mandated reporter, who in his or her professional capacity, or within the scope of his or her employment, has observed or has knowledge of an incident that reasonably appears to be abuse or neglect, or is told by an elder or dependent adult that he or she has experienced behavior constituting abuse or neglect, or reasonably suspects that abuse or neglect occurred, shall complete form SOC 341, "Report of Suspected Dependent Adult/Elder Abuse" for each report of known or suspected instance of abuse (physical abuse, sexual abuse, financial abuse, abduction, neglect (self-neglect), isolation, and abandonment) involving an elder or dependent adult.

Reporting shall be completed as follows:

- If the abuse occurred in a Long-Term Care (LTC) facility (as defined in WIC Section 15610.47) and resulted in serious bodily injury (as defined in WIC Section 15610.67), report by telephone to the local law enforcement agency immediately and no later than two (2) hours after observing, obtaining knowledge of, or suspecting physical abuse. Send the written report to the local law enforcement agency, the local Long-Term Care Ombudsman Program (LTCOP), and the appropriate licensing agency (for long-term health care facilities, the California Department of Public Health; for community care facilities, the California Department of Social Services) within two (2) hours of observing, obtaining knowledge of, or suspecting physical abuse.
- If the abuse occurred in a LTC facility, was physical abuse, but did not result in serious bodily injury, report by telephone to the local law enforcement agency within 24 hours of observing, obtaining knowledge of, or suspecting physical abuse. Send the written report to the local law enforcement agency, the local LTCOP, and the appropriate licensing agency (for long-term health care facilities, the California Department of Public Health; for community care facilities, the California Department of Social Services) within 24 hours of observing, obtaining knowledge of, or suspecting physical abuse.
- If the abuse occurred in a LTC facility, was physical abuse, did not result in serious bodily injury, and was perpetrated by a resident with a physician's diagnosis of dementia, report by telephone to the local law enforcement agency or the local LTCOP, immediately or as soon as practicably possible. Follow by sending the written report to the LTCOP or the local law enforcement agency within 24 hours of observing, obtaining knowledge of, or suspecting physical abuse.
- If the abuse occurred in a LTC facility, and was abuse other than physical abuse, report by telephone to the LTCOP or the law enforcement agency immediately or as soon as practicably possible. Follow by sending the written report to the local law enforcement agency or the LTCOP within two working days.

- If the abuse occurred in a state mental hospital or a state developmental center, mandated reporters shall report by telephone or through a confidential internet reporting tool (established in WIC Section 15658) immediately or as soon as practicably possible and submit the report within two (2) working days of making the telephone report to the responsible agency as identified below:
 - If the abuse occurred in a State Mental Hospital, report to the local law enforcement agency or the California Department of State Hospitals.
 - If the abuse occurred in a State Developmental Center, report to the local law enforcement agency or to the California Department of Developmental Services.
- For all other abuse, mandated reporters shall report by telephone or through a confidential internet reporting tool to the adult protective services agency or the local law enforcement agency immediately or as soon as practicably possible. If reported by telephone, a written or an Internet report shall be sent to adult protective services or law enforcement within two working days.

PENALTY FOR FAILURE TO REPORT ABUSE

Failure to report abuse of an elder or dependent adult is a MISDEMEANOR CRIME, punishable by jail time, fine or both (WIC Section 15630(h)). The reporting duties are individual, and no supervisor or administrator shall impede or inhibit the reporting duties, and no person making the report shall be subject to any sanction for making the report (WIC Section 15630(f)).

CONFIDENTIALITY OF REPORTER AND OF ABUSE REPORTS

The identity of all persons who report under WIC Chapter 11 shall be confidential and disclosed only among APS agencies, local law enforcement agencies, LTCOPs, California State Attorney General Bureau of Medi-Cal Fraud and Elder Abuse, licensing agencies or their counsel, Department of Consumer Affairs Investigators (who investigate elder and dependent adult abuse), the county District Attorney, the Probate Court, and the Public Guardian. Confidentiality may be waived by the reporter or by court order. Any violation of confidentiality is a misdemeanor punishable by jail time, fine, or both (WIC Section 15633(a)).

DEFINITIONS OF ABUSE

Physical abuse means any of the following: (a) Assault, as defined in Section 240 of the Penal Code; (b) Battery, as defined in Section 242 of the Penal Code; (c) Assault with a deadly weapon or force likely to produce great bodily injury, as defined in Section 245 of the Penal Code; (d) Unreasonable physical constraint, or prolonged or continual deprivation of food or water; (e) Sexual assault, that means any of the following: (1) Sexual battery, as defined in Section 243.4 of the Penal Code; (2) Rape, as defined in Section 261 of the Penal Code; (3) Rape in concert, as described in Section 264.1 of the Penal Code; (4) Spousal rape, as defined in Section 262 of the Penal Code; (5) Incest, as defined in Section 285 of the Penal Code; (6) Sodomy, as defined in Section 286 of the Penal Code; (7) Oral copulation, as defined in Section 288a of the Penal Code; (8) Sexual penetration, as defined in Section 289 of the Penal Code; or (9) Lewd or lascivious acts as defined in paragraph (2) of subdivision (b) of Section 288 of the Penal Code; or (f) Use of a physical or chemical restraint or psychotropic medication under any of the following conditions: (1) For punishment; (2) For a period beyond that for which the medication was ordered pursuant to the instructions of a physician and surgeon licensed in the State of California, who is providing medical care to the elder or dependent adult at the time the instructions are given; or (3) For any purpose not authorized by the physician and surgeon (WIC Section 15610.63).

Serious bodily injury means an injury involving extreme physical pain, substantial risk of death, or protracted loss or impairment of function of a bodily member, organ, or of mental faculty, or requiring medical intervention, including, but not limited to, hospitalization, surgery, or physical rehabilitation (WIC Section 15610.67).

Neglect (a) means either of the following: (1) The negligent failure of any person having the care or custody of an elder or a dependent adult to exercise that degree of care that a reasonable person in a like position would exercise; or (2) The negligent failure of an elder or dependent adult to exercise that degree of self care that a reasonable person in a like position would exercise. (b) Neglect includes, but is not limited to, all of the following: (1) Failure to assist in personal hygiene, or in the provision of food, clothing, or shelter; (2) Failure to provide medical care for physical and mental health needs. No person shall be deemed neglected or abused for the sole reason that he or she voluntarily relies on treatment by spiritual means through prayer alone in lieu of medical treatment; (3) Failure to protect from health and safety hazards; (4) Failure to prevent malnutrition or dehydration; or (5) Failure of an elder or dependent adult to satisfy the needs specified in paragraphs (1) to (4), inclusive, for himself or herself as a result of poor cognitive functioning, mental limitation, substance abuse, or chronic poor health (WIC Section 15610.57).

Financial abuse of an elder or dependent adult occurs when a person or entity does any of the following: (1) Takes, secretes, appropriates, obtains, or retains real or personal property of an elder or dependent adult for a wrongful use or with intent to defraud, or both; (2) Assists in taking, secreting, appropriating, obtaining, or retaining real or personal property of an elder or dependent adult for a wrongful use or with intent to defraud, or both; or (3) Takes, secretes, appropriates, obtains, or retains, or assists in taking, secreting, appropriating, obtaining, or retaining, real or personal property of an elder or dependent adult by undue influence, as defined in Section 15610.70 (WIC Section 15610.30(a)).

Abandonment means the desertion or willful forsaking of an elder or a dependent adult by anyone having care or custody of that person under circumstances in which a reasonable person would continue to provide care and custody (WIC Section 15610.05).

Isolation means any of the following: (1) Acts intentionally committed for the purpose of preventing, and that do serve to prevent, an elder or dependent adult from receiving his or her mail or telephone calls; (2) Telling a caller or prospective visitor that an elder or dependent adult is not present, or does not wish to talk with the caller, or does not wish to meet with the visitor where the statement is false, is contrary to the express wishes of the elder or the dependent adult, whether he or she is competent or not, and is made for the purpose of preventing the elder or dependent adult from having contact with family, friends, or concerned persons; (3) False imprisonment, as defined in Section 236 of the Penal Code; or (4) Physical restraint of an elder or dependent adult, for the purpose of preventing the elder or dependent adult from meeting with visitors (WIC Section 15610.43).

Abduction means the removal from this state and the restraint from returning to this state, or the restraint from returning to this state, of any elder or dependent adult who does not have the capacity to consent to the removal from this state and the restraint from returning to this state, or the restraint from returning to this state, as well as the removal from this state or the restraint from returning to this state, of any conservatee without the consent of the conservator or the court (WIC Section 15610.06).

AS AN EMPLOYEE OR VOLUNTEER OF THIS FACILITY, YOU MUST COMPLY WITH THE DEPENDENT ADULT AND ELDER ABUSE REQUIREMENTS, AS STATED ABOVE. IF YOU DO NOT COMPLY, YOU MAY BE SUBJECT TO CRIMINAL PENALTY. IF YOU ARE A LONG-TERM CARE OMBUDSMAN, YOU MUST COMPLY WITH FEDERAL AND STATE LAWS, WHICH PROHIBIT YOU FROM DISCLOSING THE IDENTITIES OF LONG-TERM RESIDENTS AND COMPLAINANTS TO ANYONE UNLESS CONSENT TO DISCLOSE IS PROVIDED BY THE RESIDENT OR COMPLAINANT OR DISCLOSURE IS REQUIRED BY COURT ORDER (Title 42 United States Code Section 3058g(d)(2); WIC Section 9725).

I, _____, have read and understand my responsibility to report known or suspected abuse of dependent adults or elders. I will comply with the reporting requirements.

SIGNATURE

DATE

CA Theft and Loss

..... Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens seeks to maintain a safe and secure Community living environment. In regard to this Theft and Loss policy, communities must implement the policy according to state regulations.

Guidelines

Although Merrill Gardens is not responsible for resident belongings, we will make an effort to safeguard their possessions.

Documentation of Theft or Loss

- Resident property reported as stolen or lost shall be documented on an *Incident Report*, regardless of the value. California requires documentation of lost and stolen property with a value of \$25 or more within 72 hours of the discovery of the loss or theft.
- When the General Manager has reason to believe resident property with a then current value of \$100 or more has been stolen, they must make a report to local law enforcement within 36 hours.
- Assist resident in searching for missing item in their apartment.
- Offer to assist the resident with filing a police report.
- Notify state agencies as required.

Identification

Residents are encouraged to identify their personal property with a distinctive mark or engraving.

Security

Residents are advised to lock their apartments when they are not present. Residents are encouraged to obtain private renter's insurance to protect their belongings.

Team members are not permitted to use master keys to enter an apartment, except in the event of an emergency or potential emergency, or with permission of the resident. The exception is if such entry, without the resident present, is documented in advance, or by obtaining prior approval.

Personal Property Inventory

An initial personal property inventory is to be completed by a team member and the resident, or the resident's representative. If the resident or representative declines to complete the inventory this will be noted on the inventory form and retained with the resident records. The Community will not be responsible for items that have not been requested to be included in the inventory or have been deleted from it.

- The inventory is to be written in ink, signed by the team member and resident, or resident's representative and dated.
- All additions to the inventory are to be made in ink and documented as above.
- Items brought into or removed from the community will be added to or deleted from the inventory by the community at the written request of the resident, the resident's family, a responsible party or a person acting on the behalf of the resident.



CA-Toxic Chemical Storage

Resident Name:

Apt. Number:

Dear Dr. _____

Regarding your patient named above, who is a resident at
{insert community name}, a licensed Residential Care Facility for the Elderly.

According to Title 22, we need your approval for your patient to self-store the following items:

- Lysol and like items
- Floor wax and like items
- Bleach and like items
- Windex and like items
- Furniture polish and like items
- Nail polish remover and like items
- Dishwashing soap and like items
- Laundry soap and like items
- Room freshener and like items

☐ Yes, the above-named resident is able to store & use the above-named items. He/She is cognitively aware to know if the above-named items are swallowed or used as undirected, harm could occur.

☐ No, the above-named resident should not have any of the listed items stored in their apartment.

Physician Signature:

Printed Name:

Date:

